

The Health Transformation Review Platform — Comprehensive Slide Deck

Audience: New users, prospective subscribers, partners, demo audiences. **Length:** ~60 slides. Each slide = one --- separator. Speaker notes follow each slide as a blockquote. **Format:** Markdown. Paste into Marp, Slidev, Pandoc → Beamer, Google Slides (one section per slide), or Keynote.

Slide 1 — Title

The Health Transformation Review

The intelligence platform for healthcare transformation leaders.

Policy • Economics • Technology • Clinical • Equity • Operations

Speaker notes: Open by naming who this is for. HTR is built for the people running, regulating, financing, building, or studying America's health system — not for casual readers. The six pillars are the unit of analysis throughout.

Slide 2 — Why HTR exists

American healthcare is not just expensive — it is **structurally failing**.

- \$4.5 trillion spent annually
- Worst outcomes among wealthy nations
- Provider burnout at record levels
- States running parallel reform experiments with limited cross-learning

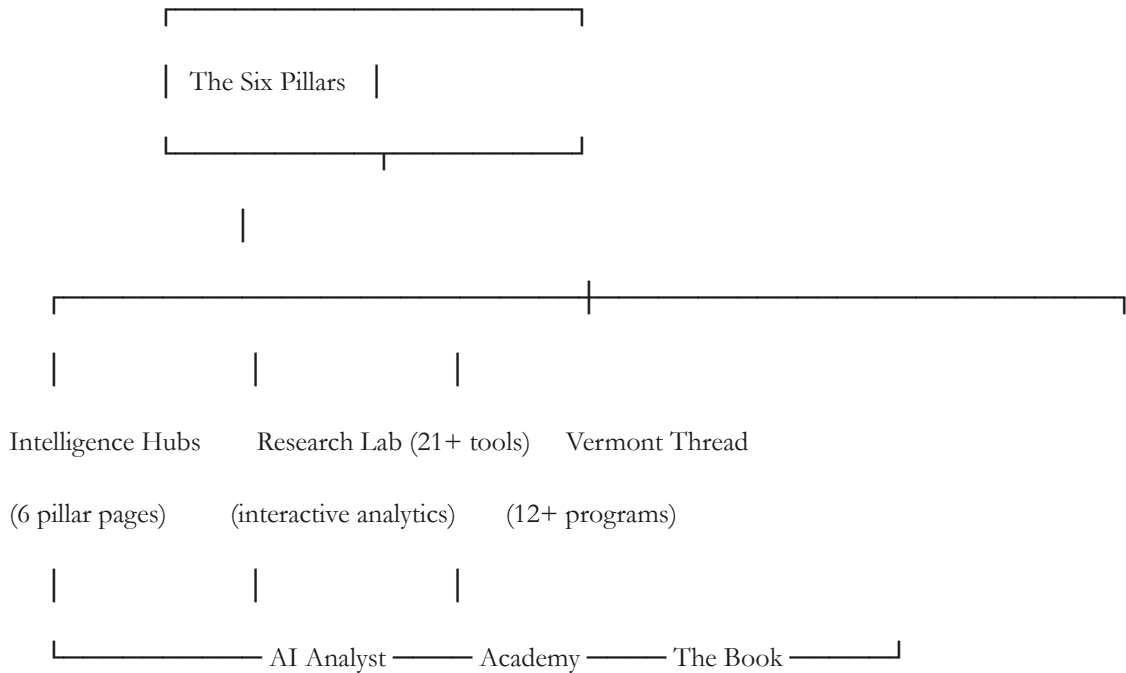
Most existing platforms treat health policy, finance, technology, clinical care, equity, and operations as **separate beats**. HTR treats them as **one connected system** — that's the core thesis.

Slide 3 — The Six-Pillar Framework

Pillar	What it covers
 Policy	Regulation, mandates, comparative & feasibility analysis
 Economics	VBC models, market dynamics, investment, CEA
 Technology	AI, digital health, interoperability, governance
 Clinical	Hospital-at-Home, precision, virtual care, genomics, pop. health
 Equity	SDOH, algorithmic bias, access disparity
 Operations	Revenue cycle, workforce, compliance, supply chain

Every page, tool, and feed on the platform is tagged to one of these six. The book uses the same taxonomy.

Slide 4 — The platform in one diagram



Three primary "modes": **Read** the intelligence, **Use** the tools, **Learn** through Academy and the book. The AI Analyst threads them together.

Slide 5 — Who HTR is for

Four reader profiles drive everything we build:

-  **Policy Professional** — state agency staff, legislative analysts, federal liaisons
-  **Healthcare Executive** — CFOs, COOs, hospital boards, ACO leaders
-  **Vermont Practitioner** — Blueprint teams, DAs, hospital ops, AHS staff
-  **Student or Researcher** — graduate students, fellows, faculty

Each profile gets a curated reading order through the book *and* a recommended starting point on the platform. See [/book](#) for the explicit map.

Slide 6 — The intellectual foundation

Every concept in the platform traces to a chapter in the book "**Transforming American Healthcare — A Six-Pillar Framework for System Transformation**" (v28, 380 pages, 20 chapters).

The platform = the **living, interactive** counterpart to the book.

The book gives you the theory and evidence; the platform gives you the data, simulators, and tracking. Either one alone is incomplete.

Slide 7 — Platform architecture (high level)

- **Frontend:** Next.js 16 + React 19 + Tailwind 4
- **CMS:** Sanity (reports, courses, webinars, tickers, analyst notes)
- **Auth & DB:** Supabase (user roles, bookmarks, comments, feedback)
- **AI Brain:** FastAPI on Railway — hybrid BM25 + vector retrieval, FlashRank reranker
- **Hosting:** Vercel
- **Payments:** Stripe (tiered subscriptions)
- **Monitoring:** Sentry + Vercel Web Vitals

Not a slide for casual viewers — keep this one for technical demos / RFP responses / board.

Slide 8 — Information architecture

The home sidebar mirrors the framework:

1. The Book (pinned)
2. Six Pillars (each opens to Intelligence Hub + Research Lab tools)
3. Academy (learning paths, courses, webinars, glossary)
4. Tools (simulators, dashboards, the Wire, indices)
5. States & Programs (Vermont 12, Oregon, California, all states)
6. Advisory & Services
7. My Library (saved items)

Multi-open accordion. The pillar always opens to its hub page; lab tools live inside.

Slide 9 — The Pillar Hubs (1/6) — Policy

/policy — Regulation, mandates, global comparison, feasibility.

- **Linked tools:** Policy Simulator, Work Requirements Calculator, H.R. 1 Cliff Scenario, Innovation Leaderboard
- **Book companion:** Chapters 4–5 (Policy Pillar)
- **Vermont thread:** Acts 167 & 68, AHEAD, Vermont legislative resources

Policy is the architectural pillar — it sets the rules everything else operates inside. Vermont is the most legislatively complete case in the country.

Slide 10 — The Pillar Hubs (2/6) — Economics

/economics — Value-based care, markets, finance, investment, CEA.

- **Tools:** APM Design Lab, Shared Savings Calculator, CEA Calculator, Global Budget Transition Modeler, Hospital Financial Stress Test, HTA Studio, Actuarial Lab
- **Book companion:** Chapters 8–9 (Economics Pillar)
- **Vermont thread:** Global budgets, AHEAD integration, hospital profiles

Economics is the most tool-dense pillar — payment reform requires modeling, not opinion.

Slide 11 — The Pillar Hubs (3/6) — Technology

/technology — AI/ML, digital health, security & governance, workflow.

- **Tools:** FHIR Interoperability Lab, Clinical Data Exchange Lab, AI Clinical Governance Lab, Digital Health Lab
- **Book companion:** Chapters 6–7 (Technology Pillar)

- **Vermont thread:** VHCURES, VITL, Act 62 HIE governance, statewide EHR feasibility

The pillar that must precede Economics — you cannot pay for value you cannot measure, and you cannot measure without infrastructure.

Slide 12 — The Pillar Hubs (4/6) — Clinical

/clinical — Hospital-at-Home, precision medicine, virtual care, genomics, population health.

- **Tools:** Risk Stratification Engine, Risk Stratification Methodology, VBC Quality Measures, High vs. Low Value Care, Clinical Quality Optimizer, Workforce Modeler
- **Book companion:** Chapters 10–11 (Clinical Pillar)
- **Vermont thread:** Blueprint for Health, VCCI, SASH, Designated Agencies

15 years of Blueprint evidence makes Vermont the most documented clinical-transformation laboratory in the country.

Slide 13 — The Pillar Hubs (5/6) — Equity

/equity — SDOH integration, algorithmic bias, access disparity.

- **Tools:** Population Health Modeler, Health Equity Studio
- **Book companion:** Chapters 12–13 (Equity Pillar)
- **Vermont thread:** Vermont SDOH (8 domains), 2-1-1 Vermont, community action agencies

Equity is treated as a structural variable, not a downstream filter. SDOH screening is built into risk stratification, not bolted on.

Slide 14 — The Pillar Hubs (6/6) — Operations

/operations — Revenue cycle, workforce, compliance, supply chain, payer ops.

- **Tools:** Transformation Scorecard, VBC Readiness Assessment, Evidence Library, Research Workspace
- **Book companion:** Chapters 14–15 (Operations Pillar)
- **Vermont thread:** Hospital profiles, GMCB reports, HCC coding strategy

Operations is where strategy meets payroll. Most transformation programs die here — they design a clinical model and forget the revenue cycle.

Slide 15 — The Research Lab (overview)

/research-lab — 21+ interactive analytical tools, grouped into five workbenches:

1. Policy & Quality (4 tools)
2. Payment Models (4 tools)
3. Interoperability & Risk (2 tools)
4. Technology & AI (2 tools)
5. VBC, Clinical & Quality (4 tools)
6. Population & Equity (2 tools)
7. Knowledge & Workspace (4 tools)

The Lab is the interactive counterpart to the book. Every concept in the book → a tool that lets you run the numbers.

Slide 16 — Research Lab — Policy & Quality bench

- **Policy Simulator** — model legislative scenarios and downstream effects
- **Medicaid Work Requirements Calculator** — H.R. 1-aware modeling
- **H.R. 1 Cliff Scenario** — coverage loss & cost-shift projections
- **Innovation Leaderboard** — state-by-state policy innovation tracking
- **Clinical Quality Optimizer** — HEDIS & quality-measure strategy

Used for: testimony prep, agency briefing decks, state-comparison analyses.

Slide 17 — Research Lab — Payment Models bench

- **APM Design Lab** — design and stress-test alternative payment models
- **Shared Savings Calculator** — ACO and provider arrangement modeling
- **CEA Calculator** — cost-effectiveness analysis
- **Global Budget Transition Modeler** — hospital prep for global budgets

Used for: AHEAD readiness, Act 68 implementation, ACO negotiation, bundled-payment design.

Slide 18 — Research Lab — Technology & AI bench

- **FHIR Interoperability Lab** — interoperability scoring + Cures Act readiness
- **Clinical Data Exchange Lab** — HL7 v2 → FHIR migration paths

- **AI Clinical Governance Lab** — the AI lifecycle: validation, deployment, monitoring
- **Digital Health Lab** — RPM, telehealth, app evaluation

Used for: HIE governance decisions, AI procurement, vendor due diligence, Act 62 implementation.

Slide 19 — Research Lab — VBC, Clinical & Quality bench

- **Risk Stratification Engine** — composite risk tiering with SDOH overlay
- **Risk Stratification Methodology** — VCCI-aligned (CDPS + utilization + SDOH)
- **VBC Quality Measures** — HEDIS, MIPS, CMS Stars walk-throughs
- **High vs. Low Value Care** — Choosing Wisely + Vermont-specific metrics

Used for: identifying the top 5% who drive 50% of cost; structuring case management; HEDIS rate strategy.

Slide 20 — Research Lab — Population & Equity bench

- **Population Health Modeler** — population segmentation, intervention design
- **Health Equity Studio** — HEDIS equity stratification, HEROI

Used for: Section 1557 compliance, MCO equity reporting, community needs assessment.

Slide 21 — Research Lab — Knowledge & Workspace bench

- **Transformation Scorecard** — composite progress scoring across the six pillars
- **VBC Readiness Assessment** — six-domain organizational readiness
- **Evidence Library** — curated, citation-grade evidence per topic
- **Research Workspace** — bring your own data, model it against HTR's frameworks
- **Innovation Leaderboard** — state-by-state ranking
- **Workforce Modeler** — staffing scenarios under transformation

These are the "synthesis" tools — pulling pillars together rather than going deep on one.

Slide 22 — The Wire — real-time intelligence feed

/the-wire — Curated stream of healthcare transformation signals.

- Pillar-tagged headlines (Policy / Economics / Technology / Clinical / Equity / Operations)
- Vermont-specific filter for state-focused readers
- Daily editorial cadence with the AI Analyst's contextual commentary

Think of The Wire as the "Bloomberg terminal" component — but for transformation, not markets.

Slide 23 — HTR Simulator

/htr-simulator — Score your organization across all six pillars simultaneously.

Input: organizational profile (hospital, ACO, state agency, payer) **Output:** pillar-by-pillar gap analysis + prioritized action sequence

This is the interactive implementation of Chapter 1's six-pillar scoring framework. Brings the framework from theory to a 30-minute self-assessment.

Slide 24 — Transformation Friction Index

/transformation-friction-index — Maps the tension between policy complexity and operational readiness.

- Scores friction along **policy design** and **operational execution** axes simultaneously
 - Identifies where implementation is most likely to stall
 - Useful for: program design, RFP scoping, post-mortem analysis of failed initiatives (e.g., OneCare Vermont)
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Slide 25 — Investment Tracker

/investment-tracker — Real-time monitoring of:

- M&A activity
- Private equity flows
- Provider/health-system consolidation
- Digital-health funding rounds

Tied directly to Chapter 9 (Economics Pillar in Practice). Used by Advisory clients for market positioning analysis.

Slide 26 — HTI Dashboard

[/hti-dashboard](#) — **Health Transformation Index** — composite scorecard across the six pillars at the state and system level.

- Updated quarterly using GMCB, CMS, and HTR proprietary data
- Trend lines for each pillar
- Drill-down to individual programs and hospitals

The HTI is HTR's "S&P 500 for transformation" — one number per state, decomposable.

Slide 27 — Medicaid Eligibility Simulator

[/medicaid-eligibility-simulator](#) — Five-step Vermont Medicaid screening for practitioners, community health workers, and benefits navigators.

- Income thresholds
- Categorical eligibility
- MAGI rules
- Work requirement applicability under H.R. 1

Designed in collaboration with community action agencies. The kind of tool the book argues should be **standard** at every clinical front door under whole-person care.

Slide 28 — Impact Simulation

[/impact-simulation](#) — System-level scenario modeling. Run a transformation initiative through 5–10-year cost, quality, and equity projections.

Where the six pillars interact. Built for cabinet briefings, board strategy sessions, and academic case studies.

Slide 29 — The Vermont Thread (overview)

Vermont is the book's primary teaching case for a reason: it is the most **legislatively complete** transformation in the United States.

12+ Vermont program pages on the platform:

- Vermont Medicaid · Blueprint for Health · VCCI · SASH · Designated Agencies · SDOH & Social Services
- Vermont Act 167 (2022) · Vermont Act 68 (2025) · Act 68 Simulator
- AHEAD Model · RHT Program (\$195M) · Vermont Hospital Profiles · Bed Capacity & Transfer
- Vermont Legislative Resources Library

If you can model Vermont, you can model anywhere — the book's claim.

Slide 30 — Vermont Act 167 (2022) & the Oliver Wyman Report

[/vermont-act-167](#) — The diagnostic mandate. The legislative directive that produced the Oliver Wyman System Redesign Blueprint.

- Mandated comprehensive system analysis
- Identified the rural hospital sustainability crisis
- Set the agenda Act 68 (2025) operationalized

Without Act 167's diagnostic foundation, Act 68 would have been politically impossible.

Slide 31 — Vermont Act 68 (2025)

[/vermont-act-68](#) — The operational mandate. Vermont's mandatory global budget legislation.

- Hospital global budgets
- Reference-based pricing architecture
- AHEAD model integration
- Implementation timeline through 2030

Plus: **Act 68 Simulator** for organizations modeling their position under the new payment regime.

The single most important state-level health policy of 2025 nationally. Read with Chapter 8.

Slide 32 — Vermont Blueprint for Health

[/vermont-blueprint](#) — 15 years of evidence. Vermont's foundational primary care transformation.

- 128 Patient-Centered Medical Home practices

- Community Health Teams in every community
- Multi-Disciplinary Team model
- The clinical infrastructure that VCCI risk stratification and SASH care coordination depend on

Read with Chapter 10.

Slide 33 — Vermont VCCI

`/vermont-vcci` — The Vermont Chronic Care Initiative.

- Targets the top 5–15% of Medicaid members by predicted cost and complexity
- Multi-domain risk stratification: utilization, CDPS, SDOH, care access
- Dedicated case management for the highest-risk tier
- Interactive: **VCCI Risk Stratification Lab**

The operational implementation of the risk-stratification logic in Chapter 10.

Slide 34 — Vermont SASH Program

`/vermont-sash` — Support and Services at Home. Housing-based care coordination.

- 200+ affordable housing communities, all 14 Vermont counties
- 13,000+ older Vermonters served
- Documented reductions in hospitalization, ED visits, nursing home placement
- Vermont's proof of concept for the **care-where-people-live** model

Read with Chapter 10's aging-population section.

Slide 35 — Vermont Designated Agencies

`/vermont-designated-agencies` — 11 regional non-profits providing community mental health, SUD, and developmental disability services.

- Howard Center (Chittenden), Washington County Mental Health, NKHS, and 8 others
- Statutory service obligation
- The institutional anchor for Vermont's behavioral health transformation

Sustainability of the DA system is the unsung dependency of the Hub-and-Spoke SUD model.

Slide 36 — Cross-state — Oregon CCO 3.0

[/oregon-cco](#) — The closest national analogue to Vermont's model.

- Third-generation Coordinated Care Organizations (2025–2030)
- Voluntary global budgets, equity accountability, community advisory boards
- A decade longer track record than Vermont
- Where Vermont's governance architecture is heading

Vermont lacks the equity accountability framework Oregon has matured. The book argues Vermont needs it by 2028.

Slide 37 — Cross-state — California CalAIM

[/california-calaim](#) — \$6.7 billion Medi-Cal transformation. 15 million members. 57 counties.

- **Enhanced Care Management (ECM)** for highest-need populations
- **Community Supports** — non-medical services as covered alternatives (housing, food, recuperative care)
- The whole-person care model at a scale Vermont's 647,000 population cannot generate

The 2026 outcome data will be the national evidence base Vermont's transformation depends on.

Slide 38 — The Academy

[/academy](#) — Structured learning paths for the framework.

- **Personalized Learning** — adaptive role-based recommendations
- **Learning Tracks** — guided sequences across pillars
- **Courses + Webinars + Case Studies**
- **Glossary** — 500+ terms
- **Medicaid Learning Center**
- **Faculty** — contributing experts

Designed for organizations bringing whole teams up to transformation literacy. Earn CME/CEU via Advisory programs.

Slide 39 — Personalized Learning

[/academy/personalized-learning](#) — Adaptive tracks based on:

- Role (executive, policy professional, clinician, researcher)
- Prior knowledge
- Goals (e.g, "implement risk stratification," "evaluate APM contracts")
- Time commitment

Backed by the AI Analyst — it tracks what you've read and recommends what's next.

Slide 40 — The AI Analyst

Two surfaces:

- **Right sidebar** — quick Q&A while reading
- [/chat](#) — full conversation, longer answers, cross-page references

What it knows:

- The entire book (380 pages, indexed)
- The Sanity content library (reports, webinars, courses)
- Vermont program pages and tool documentation
- Recent Wire items

Always cites sources. Never invents Vermont policy detail.

Slide 41 — How the AI Analyst works

You ask a question

↓

Pillar + page context attached

↓

Hybrid retrieval (BM25 + vector) across HTR corpus

↓

FlashRank reranker selects best chunks

↓

LLM composes answer + citations



Streamed to UI with inline source links

The retrieval pipeline is the secret sauce — it's why answers are grounded in HTR's own corpus, not the open web.

Slide 42 — Voice mode

Press **⌘+V** (or **Ctrl+Shift+V**).

- Voice activates speech recognition
- Spoken commands route to navigation ("Open Policy Pillar")
- Spoken questions inject into the AI Analyst
- Replies are read aloud (TTS)

Built for hands-free use — driving between hospital visits, walking between meetings, accessibility.

Slide 43 — The Wire feed mechanics

- Editorial team curates signals daily
- Each item is pillar-tagged and Vermont-tagged
- AI Analyst adds contextual commentary
- Comments enabled for subscribers

The Wire is also a data source for the Investment Tracker and HTI Dashboard — not a stand-alone feed.

Slide 44 — Bookmarks & Library

/saved — **My Library** collects what you bookmark across the platform.

- Bookmark any article, tool snapshot, or chapter reference
- Tag by project
- Export to PDF (Save-to-PDF buttons available across pages)

Roadmap: cross-device sync via Supabase (currently local-only).

Slide 45 — Subscription tiers

Tier	Access
Free	Pillar overviews, basic Wire, glossary
Subscriber	Full Research Lab, Personalized Learning, AI Analyst, full Wire archive
Student	Subscriber + academic pricing
Professional	Subscriber + advisory office hours, custom briefings
Advisory	Full custom engagement — consulting, audits, training
Admin	Internal

Stripe-managed. Switch tiers anytime. Roles are hierarchical — every higher tier inherits lower-tier access.

Slide 46 — Advisory & Services

/advisory — The professional-services arm:

- **Strategic Consulting** — six-pillar transformation engagements
 - **Custom Research** — bespoke studies leveraging HTR's evidence base
 - **Financial Audit** — hospital and ACO financial stress-testing
 - **Regulatory Counsel** — Act 167/68/AHEAD readiness
 - **IT Consulting** — interoperability, AI governance
 - **Training & Education** — team-wide transformation literacy
 - **Independent Review** — third-party assessments
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Slide 47 — HTR Connect

/connect — Member directory and community space.

- Find practitioners by pillar specialty and state
- Vermont-specific community for in-state implementers
- Discussion threads on transformation topics

Network effects: the more domain experts on HTR, the more useful the directory.

Slide 48 — Multimedia & Trending Topics

[/multimedia](#) — Podcasts, video explainers, conference recordings. [/trending-topics](#) — What the corpus + Wire say is moving right now.

Two ways to "skim" the platform when you don't have a specific question.

Slide 49 — How a Policy Professional uses HTR

Monday morning routine:

1. Open [/the-wire](#) — scan overnight signals
2. Click into a state policy headline → land on the relevant pillar page
3. Open AI Analyst → ask "How does this compare to Vermont Act 68?"
4. Use the Policy Simulator → model the proposal
5. Bookmark to [/saved](#) for next week's briefing

Vertical depth (Vermont) + horizontal breadth (50 states) is the value prop.

Slide 50 — How a Healthcare Executive uses HTR

Quarterly planning:

1. Run the **HTR Simulator** for your organization
2. Open the **Hospital Financial Stress Test**
3. Cross-check with the **VBC Readiness Assessment**
4. Use the **APM Design Lab** to model your next contract
5. Generate a board memo with the Save-to-PDF buttons

The platform replaces 4–5 consulting engagements with self-service modeling. Save the consulting hours for the actual decisions.

Slide 51 — How a Vermont Practitioner uses HTR

Daily:

1. Vermont program pages (Blueprint, VCCI, SASH, DAs) for operational guidance
2. The Wire's Vermont filter
3. The Act 68 Simulator for organizational positioning
4. Bed Capacity & Transfer tool for real-time capacity
5. AI Analyst with [/clinical](#) or [/vermont-act-68](#) context loaded

The platform was built with Vermont in mind. Vermont practitioners get the deepest experience.

Slide 52 — How a Student / Researcher uses HTR

Research workflow:

1. Start in **Academy** to map the framework
2. **Evidence Library** for citation-grade sources
3. **Research Workspace** to model your own data against HTR frameworks
4. The book as the textbook
5. The AI Analyst to traverse the corpus

Academic pricing available. Built-in citation formatting in the Save-to-PDF buttons.

Slide 53 — Data sources behind the analytics

- GMCB hospital budget filings (Vermont)
- CMS cost reports (national)
- NASHP benchmarks
- VHCURES & VITL (Vermont health data)
- Sanity CMS (HTR editorial)
- Synthetic patient generators (for Risk Stratification Lab)
- HRSA, AHRQ, KFF, Commonwealth Fund secondary sources

Every analytic on the platform is sourced — click any number to see its provenance.

Slide 54 — Quality & integrity

- Sentry on client, server, edge — every error is tracked
- Web Vitals reporting to Vercel
- Citation-grade sources throughout
- Beta access gated for now — content is locked while editorial cadence stabilizes

- Editorial corrections logged on a future [/changelog](#)
-

Slide 55 — What's new in v28 (current release)

- **The Book** integrated as a pinned navigation item
- **From the Book** callouts on every pillar and the research lab
- **VCCI Risk Stratification Lab** rebuilt with synthetic patient scenarios
- **SASH, Designated Agencies, Legislative Resources** added as state program pages
- **California CalAIM** and **Oregon CCO 3.0** added as cross-state pages
- **Personalized Learning** moved to a standalone page

Aligned with Book v28 — the v29 book update plan is already drafted ([HTR_Book_Updates_v28.md](#)).

Slide 56 — Coming soon

- Reader Mode for chapters (no more PDF iframe)
 - Annotations & highlights for subscribers
 - Shareable simulator states (URL-encoded inputs)
 - Cross-device bookmark sync
 - [/compare-states](#) — pillar-by-pillar state comparison
 - Weekly email digest
 - Public developer API ([/api/v1/*](#))
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Slide 57 — Roadmap principles

We don't build features unless they:

1. Strengthen one of the six pillars or the connections between them
2. Reduce time-to-insight for our four reader profiles
3. Are sourced — no analytics without citations
4. Are accessible (WCAG AA)
5. Can be explained in one sentence to a CFO and one sentence to a Vermont CHT nurse

Discipline over feature-creep. The platform is a research instrument, not a SaaS feature factory.

Slide 58 — Who builds HTR

- **Editorial:** healthcare policy researchers, clinical analysts, and Vermont-based domain experts
- **Engineering:** Next.js + Python + LLM tooling
- **Advisory faculty:** practicing executives, state agency staff, academics
- **External voices:** book reviewers, guest contributors, named in [/academy/faculty](#)

Small, focused team. Vermont-headquartered. National in scope.

Slide 59 — Try it now

- Open [/about/framework](#) to see the Six-Pillar Map in motion
 - Open [/htr-simulator](#) and score yourself in 30 minutes
 - Open [/the-wire](#) to see what just happened today
 - Open [/research-lab](#) to browse all 21 tools
 - Or just open the AI Analyst (K or the right sidebar) and ask anything
-

Slide 60 — Contact & next steps

- **Web:** healthtransformationreview.com
- **Email:** advisory@healthtransformationreview.com
- **Beta access:** request via [/beta](#)
- **Subscriptions:** [/pricing](#)
- **Custom engagements:** [/advisory](#)

Thank you. Ask anything — the AI Analyst is open.

End of Platform Deck (60 slides).